Full Title: "Investigating the links between Canadian cities' social infrastructure and community health: Mapping national trends of self-reported health and behaviour, social connection, chronic conditions, and mortality"

Summary:

Lay Title:

Lay Abstract:

CCHS PUMFs | CSS PUMFs | SMHSE PUMFs | CVSD PUMFs | CENSUS PUMFs |

1 BACKGROUND & OBJECTIVES:

Inequities in social determinants of health (SDoH) like the local built environment, community safety, social networks, and access to physical activity opportunities[1] can result in differences in life expectancy up to 33 years between countries.[2] In Canada, understanding the myriad implications of SDoH differentials and disparate health outcomes is of increasing importance to governments at all levels seeking to achieve health equity and improved health trajectories for vulnerable communities.[e.g., 3,4] Illustrative examples of the links between SDoH and disparate health outcomes in Canada include area income (e.g., lower area income correlated with lower life expectancy), material and social deprivation (e.g., greater deprivation associated with higher suicide mortality), and physical and social environments (e.g., prevalence of living in below-standard housing is ~7.5x higher for Canadians in the lowest income group).[5] Leaving SDoH unaddressed with respect to environmental factors comes at significant cost for Canadians. Poor mental health, including mental illnesses, is estimated to cost $51 billion annually in health care expenditures and lost economic productivity.[6] Low levels of physical inactivity in Canada similarly has total direct and indirect annual health care costs of $4.3-$9.3 billion,[7] as well as an additional $2.2 billion in costs related to decreased work capacity and productivity losses.[8] Emerging research multiple country trends also notes rising economic and health care costs associated with social isolation; for instance, in the US, Medicare programs spend $6.7 billion USD annually to address loneliness,[9] in the UK an estimated £2.5 billion is lost in annual economic productivity due to loneliness-related issues like greater absences and turnover,[10] and in the Netherlands loneliness is associated with an indirect increase of €3.5 billion in total healthcare expenditures.[11]

An emerging potentially cost-effective and community level intervention to support the health of Canadians is addressing local SDoH, and specifically the quality of area social infrastructure (SI; i.e., places and spaces like gardens, gathering sites, libraries, parks/pools, and pubs that help build and maintain community and social life[12]). High quality SI can measurably improve different aspects of health such as resilience, social ties, and active living[13–15] as a result of allowing for people to build connections, develop healthy and active habits, and enhance collective action capacity. SI can also have important protective health benefits. For instance, everyday networking places like sites for local community organizations (e.g., community centres),[16] cafés, and places of worship can function as nodes through which individuals engage in building and strengthening social ties thereby mitigating the potential for isolation and loneliness.[17] However, SI is a relatively new concept[18] that has only recently been connected to health services[19] and recreational studies[20] topics. Moreover, international reviews of SI have reported most existing research appears to be local in scale (i.e., few national-scale analyses),[21] lacks clarity regarding which specific communities (e.g., vulnerable social identity groups) may benefit the most and in what ways from SI,[22] and is yet to directly link SI with a comprehensive set of health outcomes beyond social indicators.[23] Using the 25 largest municipalities in Canada to map national trends regarding how the quality of local SI is associated with multiple dimensions of health, the proposed research has three objectives:

O1: Explore how the quality and prevalence of local SI affects self-reported overall physical and mental health (CCHS), and the size and strength of social networks (CSS); [i.e., overall health, social networks]

O2: Investigate how the quality and prevalence of local SI predicts drug and alcohol use and smoking behaviours (CCHS), as well as engagement in healthy and help-seeking behaviours

(SMHSE); [i.e., unhealthy and healthy behaviour] and incidences of chronic conditions (SMHSE/CCHS) and causes of mortality (CVSD). [i.e., chronic conditions, health trajectories, mortality]

O3: Investigate how residents of two distinct RoW neighbourhoods regarding their SI quality (i.e., low-quality SI, high-quality SI), as determined by the measure developed in O1-O2, perceive, access, and make use of their local SI apropos of their health/illness.

2 RESEARCH METHODS & TIMELINES:

This research aligns with CIHRs Institute of Population and Public Health (IPPH) mandate, and specifically the areas structural determinants of health and public health systems, and will proceed via three sequential studies that, collectively, quantify the influence of SI as component of both the structural determinants and public health system in Canada.

2.1 SI and Overall Health, Social Support (O1): Explore SI and self-reported overall physical and mental health, and the size and strength of social networks (April 26/July 26).

2.1.1 Data Linking: Google Maps Places API + 2021 Census + 2021 CSS + 2021 CCHS

2.1.2 Variables:

Independent Variables: quality of local social infrastructure (derived from Google Maps Places API: map libraries, community centers, city hall facilities, places of worship, cafes, bookstores, parks, squares, fountains, and gardens).

Dependent Variables:

Self-reported health (CCHS): overall health assessment, life satisfaction, mental health assessment, life stress assessment, work stress assessment, difficulties with personal care, and mobility limitations.

Social networks (CSS): sense of belonging to local community, sense of belonging in Canada, someone to count on, loneliness, number of peopled trusted in neighbourhood, and total number of acquaintances.

Covariates (all in the 2021 Census): age, family structure, citizenship, dwelling types, ethnic or cultural origin, gender, highest certificate/diploma/degree, household low-income status, immigrant status, household after-tax income groups, labour force status (3), marital status

2.1.3 Data Analysis:

2.2 SI and Daily Health Behaviours, Long-Term Health (O2): Investigate SI and drug and alcohol use and smoking behaviour, engagement in healthy and help-seeking behaviours (Aug 26/Nov 26).

2.2.1 Data Linking: Google Maps Places API + 2021 Census + 2021 SMHSE & CCHS + 2021 CVSD

2.2.2 Variables:

Independent Variables: quality of local social infrastructure (derived from Google Maps Places API: map libraries, community centers, city hall facilities, places of worship, cafes, bookstores, parks, squares, fountains, and gardens).

Dependent Variables:

Alcohol Use (CCHS): ever had a drink, drink in last 12 months, frequency of drinking in last 12 months, frequency of drinking more than 4/5 drinks in last 12 months.

Drug Use (CCHS): ever used hard drugs, hard drug use in last 12 months, ever use of amphetamines/methamphetamines, amphetamines/methamphetamines use in last 12 months, ever use of intravenous drug use, intravenous drug use in last 12 months

Stressful Events (CSS): any experience of highly stressful or traumatic events, worst event experienced, age at worst event experienced, contemplated suicide, contemplated suicide as a result of worst experience, social impacts of stressful events

Healthy: Physical Activity (18+) (CCHS): physical activity in last 7 days, time spent in physical activity in last 7 days, MVPA last 7 days, time spent in MVPA in last 7 days

Healthy: Healthy Behaviours (CSS): type of activities engaged in to maintain or improve health

Healthy: Mental Health Services (CSS): any use of resources to management emotions/mental health/alcohol/drugs, type of resource used

Covariates (all in the 2021 Census): age, family structure, citizenship, dwelling types, ethnic or cultural origin, gender, highest certificate/diploma/degree, household low-income status, immigrant status, household after-tax income groups, labour force status (3), marital status

2.2.3 Data Analysis:

2.3 SI and Long-Term Health (O3): Comparative investigation of RoW residents perceptions, access, and usage of local SI regarding health/illness (Dec 26/Mar 27).

2.3.1 Data Collection:

2.3.2 Variables:

Independent Variables: quality of local social infrastructure (derived from Google Maps Places API: map libraries, community centers, city hall facilities, places of worship, cafes, bookstores, parks, squares, fountains, and gardens).

Dependent Variables: Chronic condition measures will be taken from the 2021 SMHSE and 2021 CCHS. Measures from the SMSHE will include assessments of mood disorders (e.g., depression, bipolar), anxiety disorders (e.g., OCD, panic disorder), and PTSD. Items from the CCHS will be evaluations of sleep apnea, arthritis, high blood pressure, heart disease, stroke, diabetes, cancer, Alzheimer's Disease, and chronic fatigue syndrome.

Health trajectory assessments will be assessed for both tails of the lifecourse using multiple items from the 2021 CVSD. '*Infancy and childhood*' life trajectory measures will include perinatal death of population, birth weight of infant, and health-adjusted life expectancy of person. With respect to '*late adulthood*' measures included in the analyses will be probability of survival of person, potential years of life lost of person, and potentially avoidable mortality of person.

Mortality measures will also be taken from the 2021 CVSD and will include age at time of death of person, cause of death of person (type), and premature mortality of person (type).

Covariates (all in the 2021 Census): age, family structure, citizenship, dwelling types, ethnic or cultural origin, gender, highest certificate/diploma/degree, household low-income status, immigrant status, household after-tax income groups, labour force status (3), marital status

2.3.3 Data Analysis:

3 CHALLENGES & MITIGATION PLANS:

This project will involve the creation of a national dataset quantifying the nature and quality of SI in Canada's largest municipalities. There are two important challenges to note about this task. Fist, while we plan to operationalize SI in accordance with current literature (i.e., places and spaces that help build/maintain community and social life[12]), we recognize that there are other outlines of SI that introduce features such as SI being the "system of organizations … that enable people to connect[24]" that are difficult to quantify. Unable to quantify larger, more abstract notions of 'systems' in the analysis, our approach uses a fairly broad definition of SI that makes room for the inclusion of as many important spatial influences including green and blue spaces (gardens, parks, pools, ponds), communal gathering sites (plazas, pedestrian malls, public easements), pedestrian infrastructure (trails, multi-modal paths, sidewalks), educational resources (libraries, schools), leisure sites (community centres, sports facilities), and social settings (pubs, restaurants). And second, we elected to create a database of SI for XX municipalities in Canada where the average population is XXX,XXX and spatial size is XXX.X km^2 (per publicly available StatCan information). Consequently, we recognize that our analyses are not generalizable to smaller population or more compact communities. Intending to publish our results in open access journals and provide evidence briefs on our Team's new open access policy platform (www.CivicInquire.com), we will encourage future research to use or adapt out method and explore the community typologies we were unable to investigate in this research. Other potential challenges include … Linking issues? Coarse quantifications of SI due to API?

4 RESEARCH TEAM:

NPA Leia Minaker is an Associate Professor in the School of Planning at UW. She is also the Director of the Future Cities Institute (FCI) at UW and has expertise in public health research, urban planning, epidemiological risk assessment analyses, SDoH (esp. built environment, health disparities), dataset linking, and knowledge mobilization (KMb) strategies and resource development for health and planning practitioners. She will lead the team across **O1-O3**, as well as help organize the KMb activities. **Co-I Troy Glover** is a Professor in the Department of Recreation and Leisure Studies at UW. He is also the Director of the Healthy Communities Research Network and with expertise in social infrastructure, municipal policy analysis, the social impacts of urban planning, and qualitative methods. He will help lead the qualitative components of the proposed research (**O3**) and assist in the data collection and analysis of **O1** and **O3**, and will also be involved in the recruitment for and running of the KMb activities. **Co-I Rob Feick** is an Associate Professor in the School of Planning at UW. He has expertise in methods for using geospatial information technology and data to support municipal decision making, GIS for infrastructure evaluations and assessments, and geospatial data visualization KMb tool development. He will lead the analysis and contribute to the write-up for **O1** and **O2**, as well as support the KMb activities. **Co-I Adrian Buttazzoni** is an SSHRC postdoctoral fellow in the School of Planning at UW who has expertise with health policy, community health promotion, urban design and human health/social outcomes, SDoH (esp. housing, built environment, community social well-being), and urban planning. He will assist with the project

coordination, data collection and cleaning, analysis, and writing tasks associated with **O1-O3** and support the KMb activities. **PKU Eric Avner** is the President & CEO of the Waterloo Region Community Foundation (WRCF)… **Collaborator Derek Phillips** …

5 KNOWLEDGE MOBILIZATION:

NPA Minaker is Director of the University of Waterloo's Future Cities Institute (FCI). The FCI's purpose is to foster healthy, prosperous future cities through applied, interdisciplinary research. The FCI has strong and extensive networks, including municipalities, planners, developers, and elected officials. Working closely with **PKU** Avner and **Collaborator** Phillips, the **NPA** and **Co-Is** will organize two local city sprints (i.e., group meetings for action planning, timelines). City sprints will be organized around the theme of 'building and promoting social infrastructure to support community cohesion and well-being'. Our team, which includes the Waterloo Region Community Foundation, will engage other relevant community partners (e.g., local planners, local municipal officials) and research members (e.g., healthcare system, social welfare system researchers) to participate in the two city sprints. Specifically, we use a 2-day design where both sprints are developed and facilitated by the FCI and invited partners are instructed to focus on identifying potential solutions and developing prototypes to address challenges found in **O1-O3**.

6 ANTICIPATED IMPACTS:

This will be one of the first national-level spatial analyses of the relationships between local SI quality and prevalence and a multitude of health outcomes that capture perceptions of health, daily health behaviours, and long-term chronic condition and mortality trends. Our intersectional analyses, which will explore the extent to which SI potentially uniquely affects the health of several equity-deserving groups (e.g., low-income families), will help identify how structural determinants of health (e.g., urban design) and public health system (e.g., access to services) in Canadian cities contribute to disparate assessments of individual health trajectories and, risks for unhealthy behavioural habits, and long-term health outcomes. By leveraging FCI support and our professional links associated with the WRCF, our findings will contribute to supporting equity-focused health policy recommendations for local health units and health care service providers.

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Other Stuff (From two page proposal)

Dataset Information

Discussed in group conversation: *Canadian Community Health Survey (2021) - CCHS | *Canadian Social Survey (2021) - CSS | *Canadian Census (2021) | *Survey on Mental Health and Stressful Events (2021) – SMHSE | Canadian Vital Statistics - Death database (2021) - CVSD
**2021 datasets can be linked via postal codes*

Variables of Interest (Operationalizing the DVs)

Bonding capital: various CSS, CCHS, and SMHSE items

Dataset	Variable	Item/Details
CSS 2021	Sense of belonging to local community (SBL) - SBL_Q100	How would you describe your sense of belonging to your local community? 1: Very strong 2: Somewhat strong 3: Somewhat weak 4: Very weak 5: No opinion
	Trust in different groups of people (TIP) TIP_Q10	... what is your level of trust in each of the following groups of people? People in your family
CCHS 2021	Social provisions (SPS) - Question identifier:SPS_Q035	I feel part of a group of people who share my attitudes and beliefs.
SMHSE 2021	Healthy behaviours (BH) - BH_Q35	Are you currently doing any of the following activities to maintain or improve your health? (possibly dichotomize?) 01: Communicating with friends or family 02: Communicating with health professionals (e.g., counsellor or therapist) 03: Communicating through social media 04: Meditating or seeking spiritual guidance 05: Exercising 06: Participating in hobbies (e.g., gardening, journaling or crafts) 07: Ensuring adequate sleep (e.g., avoiding screen time before sleep, avoiding caffeinated drinks or maintaining regular sleep cycle) 08: Other 09: None of the above
	PTS_Q35M – Stressful Events	Keeping your worst event in mind, over the past month, how much have you been bothered by the following problems? Feeling distant or cut off from other people

		1: Not at all 2: A little bit 3: Moderately 4: Quite a bit 5: Extremely
	SPS_Q05G – Social Provisions	Please indicate to what extent each statement describes your current relationships with other people. I feel part of a group of people who share my attitudes and beliefs 1: Strongly agree 2: Agree 3: Disagree 4: Strongly disagree
	SPS_Q05B	Please indicate to what extent each statement describes your current relationships with other people. There are people who enjoy the same social activities I do 1: Strongly agree 2: Agree 3: Disagree 4: Strongly disagree

Bridging capital: trust in different groups of people from CSS

Dataset	**Variable**	**Item/Details**
CSS 2021	TIP_Q20	… what is your level of trust in each of the following groups of people? People you work with
	TIP_Q21	… what is your level of trust in each of the following groups of people?
	TIP_Q22	… what is your level of trust in each of the following groups of people? People who speak a different language than you
	TIP_Q23	… what is your level of trust in each of the following groups of people? People with a different religion than you
	TIP_Q24	… what is your level of trust in each of the following groups of people? People with a different ethnic or cultural background than you
	TIP_Q25	… what is your level of trust in each of the following groups of people? Strangers

Trust in others, institutions, and government: trust in different groups of people AND/OR confidence in institutions from CSS

Dataset	Variable	Item/Details
CSS 2021	Trust in different groups of people (TIP) TIP_Q10	… what is your level of trust in each of the following groups of people? People in your family
	TIP_Q15	… what is your level of trust in each of the following groups of people? People in your neighbourhood
	TIP_Q20	… what is your level of trust in each of the following groups of people? People you work with
	TIP_Q21	… what is your level of trust in each of the following groups of people?
	TIP_Q22	… what is your level of trust in each of the following groups of people? People who speak a different language than you
	TIP_Q23	… what is your level of trust in each of the following groups of people? People with a different religion than you
	TIP_Q24	… what is your level of trust in each of the following groups of people? People with a different ethnic or cultural background than you
	TIP_Q25	… what is your level of trust in each of the following groups of people? Strangers
	Confidence in institutions (CII) CII_Q10	… How much confidence do you have in the following institutions? The police
	CII_Q15	… How much confidence do you have in the following institutions? The justice system and courts
	CII_Q30	… How much confidence do you have in the following institutions? The school system
	CII_Q40	… How much confidence do you have in the following institutions? The Federal Parliament

Size and strength of social network and connections: trust in neighbourhood people from CSS AND/OR number of connections from GSS

Dataset	Variable	Item/Details
CSS 2021	Trust in neighbourhood people (TNP)	How many people do you trust in your neighbourhood? 1: Most of the people 2: Many of the people 3: A few of the people 4: Nobody
		the total number of acquaintances a person has. This number excludes relatives and close friends.

Shared values: shared values from CSS

Dataset	Variable	Item/Details
CSS 2021	Shared values (SVR) SVR_Q01	... To what extent do you personally agree with the following values? Human rights
	SVR_Q02	... To what extent do you personally agree with the following values? Respect for the law
	SVR_Q03	... To what extent do you personally agree with the following values? Gender equality
	SVR_Q04	... To what extent do you personally agree with the following values? English and French as Canada's official languages
	SVR_Q05	... To what extent do you personally agree with the following values? Ethnic and cultural diversity
	SVR_Q06	... To what extent do you personally agree with the following values? Respect for Indigenous (First Nations, Métis or Inuit) culture

Perceived social support and belonging: social provisions scale from CCHS AND/OR sense of belonging from CSS

Dataset	Variable	Item/Details
CCHS 2021	Social provisions (SPS) - Question identifier:SPS_Q015	I have close relationships that provide me with a sense of emotional security and wellbeing.
	Social provisions (SPS) - Question identifier:SPS_Q020	There is someone I could talk to about important decisions in my life.
	Social provisions (SPS) - Question identifier:SPS_Q025	I have relationships where my competence and skill are recognized.

	Social provisions (SPS) - Question identifier:SPS_Q035	I feel part of a group of people who share my attitudes and beliefs.
	Social provisions (SPS) - Question identifier:SPS_Q050	There are people I can count on in an emergency.
	Work family balance (WLB) - Question identifier:WLB_Q75	My involvement in my family helps me acquire skills and this helps me be a better worker.
	Work family balance (WLB) - Question identifier:WLB_Q80	My involvement in my family puts me in a good mood and this helps me be a better worker.
	Work family balance (WLB) - Question identifier:WLB_Q85	My involvement in my family encourages me to use my work time in a focused manner and this helps me be a better worker.
CSS 2021	Sense of belonging (SBL) - Question identifier:SBL_Q100	How would you describe your sense of belonging to your local community?
	Sense of belonging (SBL) - Question identifier:SBL_Q400	How would you describe your sense of belonging to Canada?
	Trust in people (PCT) - Question identifier:PCT_Q10	Generally speaking, would you say that most people can be trusted or that you cannot be too careful in dealing with people?

Variables of Interest (Operationalizing the IVs*)

Family demographics: age, gender, ethnicity, household income, job status, educational attainment, marital status all available in the 2021 Census.